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# Relational design experiments for improved health care access for persons who sell sexual services in Trondheim

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**ABSTRACT:** Nadheim offers healthcare to persons selling sexual services. Using relational, feminist, and system-oriented design, a rich design methodology combined cultural probes, vignette studies, and giga mapping unc toovered issues of service fragmentation, stigma, and digital exclusion. A co-created digital tool offering anonymous, centralized access to health, legal, and support services. An added speculative concept imagines a sex worker union to allow for radical change. Findings highlight trust, inclusion, and co-agency, positioning design as a catalyst for social justice.

**KEYWORDS:** co-design, health services, inclusive design, design theory, relational design

## 1. Introduction

This study analyzes a design project carried out by two NTNU industrial design students in collaboration with Nadheim Trondheim, a low-threshold service by Norwegian Church Aid supporting people who sell sexual services (PSSTs). The project explored systemic challenges and developed tools to improve coordination and support for PSSTs. Using a framework grounded in social justice, feminist theory, and systemic design, the students centered user experiences and focused on rights, safety, and dignity. Participatory methods were used to counter stigma and stereotypes. The first and second author supervised the project, which informed the students' master thesis (Rutherfurd & Tvedten 2024).

Through interviews, workshops, cultural probes, and participatory activities, the project identified systemic gaps and produced 'Ly'—a conceptual digital tool offering PSSTs and service providers an anonymous, accessible overview of health, legal, and support services. Ly functions as a relational intervention to foster trust, reduce stigma, and strengthen coordination. A speculative concept, 'Yl,' imagined a sex-worker union to challenge norms and envision alternative futures. Together, these outcomes demonstrate how relational, feminist, and systemic design can reveal hidden structures, elevate marginalized voices, and support more inclusive service systems.

### 1.1. Persons who sell sexual services (PSSTs) and health care

Historically, prostitution in Norway has been framed through moral, legal, and medical lenses, from 19th-century "visitation" laws to current debates on sex work as violence versus empowerment. Feminist perspectives remain polarized, viewing sex work as either exploitation or agency. The analysis of the student project shows a relational, inclusive design approach centring PSST voices. Digitalization has increased invisibility, shifting sex work to platforms like RealEscort.eu and OnlyFans, complicating outreach and research. Since the last comprehensive study, the field remains under-researched and underfunded



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(Tveit & Skilbrei 2008). Lack of visibility limits political will, leaving PSSTs to navigate fragmented services with inconsistent access to health care, legal aid, and social support. Without inclusive design, digital tools risk excluding those with limited language skills or digital literacy (Fisk, Dean et al. 2018).

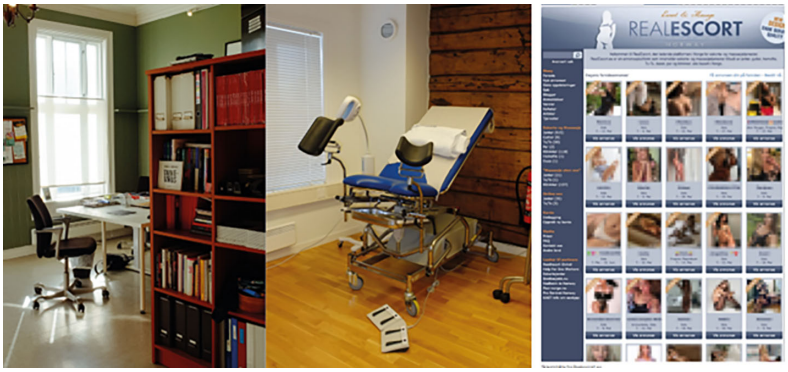



**Figure 1.** Photos from the inside of a health care clinique for persons selling sexual services. The righ image is an example of the sites that Nadheim workers use to locate women for outreach in Trondheim

Sex work in Norway operates within a complex and often hidden system shaped by law, stigma, migration, and fragmented services. Although selling sexual services is legal, the criminalization of buying them under the Norwegian Sex Purchase Act (JD 2025) pushes sex work underground and makes access to health care, legal support, and social services more difficult. Many PSSTs are low-income migrants who move between cities and countries and often lack welfare rights, language support, and legal clarity (Kock, Ranes et al.).

Nadheim, the project's main collaborator, provides low-threshold support to people with experience in selling or exchanging sexual services and to individuals affected by human trafficking. Open to anyone over 18—regardless of gender, nationality, or residency status – Nadheim offers free, anonymous services. It operates in Stavanger, Bergen, Oslo, and Trondheim, and is the only service specifically for PSSTs in Trondheim. Most visitors in Trondheim are women, though men and people of other gender identities also attend. Some drop in briefly, while others rely on the service for years. Many face health challenges, unstable income, violence, limited knowledge of available services, and significant stigma. Since 2019, Nadheim Trondheim has offered health care support, counselling, open conversations, contraceptives, STI testing, and outreach. The Trondheim team consists of two full-time positions shared among four staff, along with health personnel. The 2023 annual report indicates rising numbers of visitors, conversations, and distributed condoms compared to previous years (Bymisjon 2020).

## 2. Theory and methods

The student designers, whom also are co-authors of this paper, applied System-Oriented Design (SOD) as an overarching methodological framework to navigate the complex labour conditions and everyday experiences of PSSTs. While SOD offers a tool-oriented approach, its application was overarched by relational design: where feminist, relational, and care ethics theories guide design decisions, proposals, reflection and analysis. A relational design approach emphasizes the importance of designing with, rather than for, people – especially those in vulnerable or marginalized positions. Relational design is not merely about creating artifacts or services, but about cultivating relationships, trust, and mutual understanding through design processes (Nielsen & Bjerck 2022). It is a practice that acknowledges the emotional, social, and political dimensions of design, and seeks to foster empathy, dialogue, and co-agency between designers and participants. In the context of this design project for sex-workers safety in Norway, relationality proved valuable for navigating design decisions with care and for critically reflecting on aspects of power and privilege. In this study students evolved into a relational design framework combining feminist principles with system-oriented design, this allowed critical guidelines for engaging with a stigmatized and often invisible population. The relational turn in design further emphasizes 'designing in, with, or for relationships' (Nielsen & Bjerck 2022, Udoewa & Gress 2023, Arturo Ortega Alvarado 2024) framing co-design as an open, evolving process (St John & Akama 2022). To expand on the relational approach, the students adopted a feminist lens, framing care as a political and relational practice (Gilligan 2024). Care challenges rationalist ideals, emphasizing interdependence and reflexivity (Fisher & Tronto 1990). In practice, design students combined SOD with feminist design



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theory, where choosing tools that could center marginalized voices and promoting equity, care, and inclusion were prioritized. These frameworks guided tools and spaces that help PSSTs navigate life with dignity and agency.

## 2.1. Methodology for relational design experiments

Reviewing the project report we see that a relational approach was particularly important given the sensitivity of the topic and the vulnerability of the user group. Many PSST are migrants, some with irregular legal status, and most face significant stigma (Rutherfurd & Tvedten 2024). Relational design allowed the team to navigate these complexities, using design tools such as cultural probes, with curiosity and care, ensuring that participants felt safe, heard, and respected. Relational design was operationalized through sustained collaboration but also through an experimental and exploratory approach seeking to work with the complexities of sex work, health, safety and dignity with Nadheim in Trondheim and other actors in the service system. The designers engaged in repeated visits, informal conversations, workshops, and co-creative sessions that prioritized openness, trust, and mutual respect. Rather than extracting data, the team sought to build relationships that could inform and shape the design process and sought to understand and design for future relational values for the persons who sell sexual services and keeping an eye out for power relations and potential cognitive biases. The study adhered to ethical guidelines for working with vulnerable populations, including informed consent, confidentiality, and minimization of harm. The team also engaged actively during the design process in critical self-positioning, using tools such as autoethnography and identity mapping to reflect on their own biases, assumptions, and social positions. Inspired by Donna Haraway's concept of "situated knowledges," the designers acknowledged that their perspectives are partial and shaped by their own experiences. They spent numerous hours in discussion to reflect on their own assumptions and roles during the project. This reflexivity helped prevent epistemic traps and ensured that the design process remained open to multiple truths and lived realities.

### *2.1.1. Methods of inquiry*

The methodology employed therefore combines a relational design methodology – using co-design, workshops and interviews – with SOD tools. The goal of the students was not to impose solutions, but to co-create interventions that reflect the lived realities, needs, and aspirations of people who sell sexual services (PSST). A diverse set of qualitative and participatory methods were used to gather insights and co-create proposals including an initial desk study, review of literature, policy documents, and media narratives, followed by semi-structured interviews conducted with PSSTs, service providers, legal experts, and health professionals. Alongside these qualitative interviews, a series of small sized workshops were held with five Nadheim staff in Trondheim, two Nadheim staff in Oslo, and other stakeholders to map challenges and ideate solutions. Two workshops and interviews with seven staff members from Nadheim Trondheim and Oslo were conducted to outline the Norwegian service system surrounding PSSTs. This system is characterized by legal ambiguity, social stigma, fragmented services, and diverse user needs. Using SOD tools such as Giga mapping, actor-network diagrams, and service blueprints(Sevaldson 2011), the team mapped actors, relationships, and systemic barriers. Gigamapping, an interactive visualization process, allowed stakeholders to integrate perspectives and identify leverage points for intervention (Sevaldson 2011). To complement systemic mapping, sensitive and relational methods were employed to access PSSTs experiences and attitudes shaping stigma. Initial outreach based on criteria—PSSTs in Trondheim, using WhatsApp, selling physical services, and representing different nationalities—yielded limited responses, highlighting the difficulty of reaching this population. To address this, the team developed a **cultural probe**; Crabtree refers to situations in which 'research in these contexts is often regarded as not merely difficult but often inappropriate and intrusive', and have developed the method of Cultural Probes. Probes are 'artefacts that provide a range of materials reflecting important aspects of the participant's local cultures and, on being returned to the investigators, these reflections inspire design'. They can be diaries, photo albums, post cards or other artefacts that the marginalized persons we design for can interact with and return without us having to intervene in their sensitive life and work situations (Crabtree, Hemmings et al. 2003). The students designed journal titled 'What's Your Narrative', distributed in city locations frequented by PSSTs. This probe invited participants to record personal reflections, needs, and aspirations in a non-intrusive way, enabling richer qualitative insights (Crabtree, Hemmings et al. 2003).



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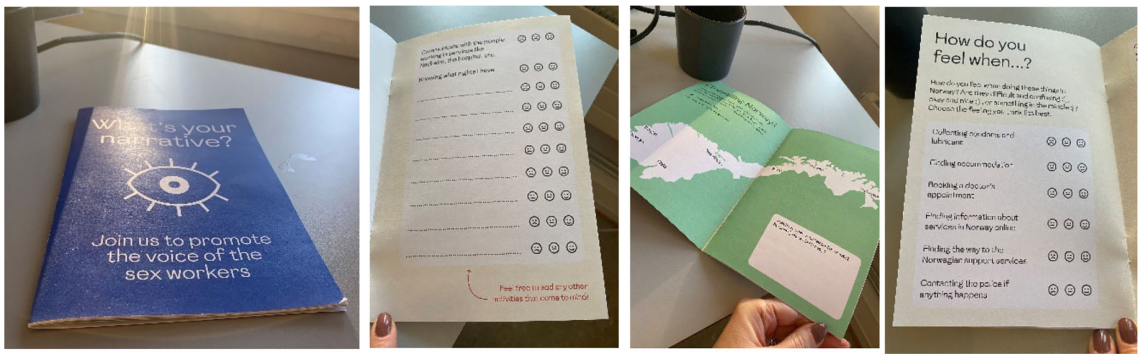



**Figure 2.** The culural probe with some of the pages illustrated for PSSTs to fill in

Alongside the cultural probe, a vignette study was created because one of the areas identified in talks with Nadheim workers, was stigma and the perspective of otherness. A **vignette study** is a manner of which to inquire about attitudes and presumptions, in a way where people will be more honest in their feedback than if asked directly about their attitudes. By providing scenarios, where the respondent can react to certain statements about the sex selling group, they could assess public attitudes toward sex work through a standardized survey instrument (APPS)(Aguinis & Bradley 2014). The scenarios were presented as a survey, and QR codes were distributed at the student campus. The survey mainly played a role for the reflection of the design team upon the role of PSSTs in society and the speculative design process. The students also conducted an **observational walk**, in which they imagined being a woman entering the city of Trondheim and later Oslo, arriving to the train station and walking out of there, occupying areas they imagined or were told PSSTs would be walking, in order to identify which places in the city could be relevant for placing communication material to reach PSSTs with information about health care services or workers' rights. Another way for designers to retrieve knowledge is through concept development and feedback. Development and testing of a digital tool for improved service access, Ly, through sketches, mock-ups, and scenario-based evaluation. Each method was chosen to balance depth of insight with ethical sensitivity. The project culminated in a co-design workshop with five representatives from Nadheim, the police, municipal services, and other stakeholders. By involving those who work directly with PSST, the designers ensured that the tool reflects real-world needs and constraints. The workshop also fostered dialogue between actors who rarely collaborate, laying the groundwork for future systemic change.

## 3. Key insights

The design process revealed a wide range of systemic, social, and individual challenges faced by people who sell sexual services (PSSTs) in Norway. The insights are organized into five thematic areas: service system fragmentation, health care access, stigma and trust, digital exclusion, and end user diversity.
The design research show that the PSST population in Norway is highly diverse, encompassing a wide range of nationalities, genders, ages, and motivations. This diversity challenges simplistic narratives and underscores the need for tailored support. Most PSSTs in Norway are migrant women, but men, trans people, and Norwegian citizens also sell sexual services. Motivations include economic necessity, empowerment, trauma, and lack of alternatives. Some work independently, while others are affiliated with agencies or networks. Needs vary widely, from legal advice and health care to emotional support and community. The design team emphasized that there is no "default user" in this context. Solutions must be flexible, adaptive, and responsive to individual circumstances.
One of the most consistent findings during the co-design process was the **lack of coordination and visibility** within the Norwegian service system providing services to persons who sell sexual services. PSST often navigate a maze of disconnected services, including health clinics, legal aid, social support, and outreach programs.
The cultural probe was returned with some peakes into the life of a PSST in Trondheim, for example explaining the lack of hygiene, and the need for a place to register clients:

> *Every client is quite discusting, because they smell bad, a majority has poor hygiene.*
>
> *It would be nice with one portal, in which every worker can register . . . and also register numbers of suspicious clients'*

Spanish speaking person who sells sexual services



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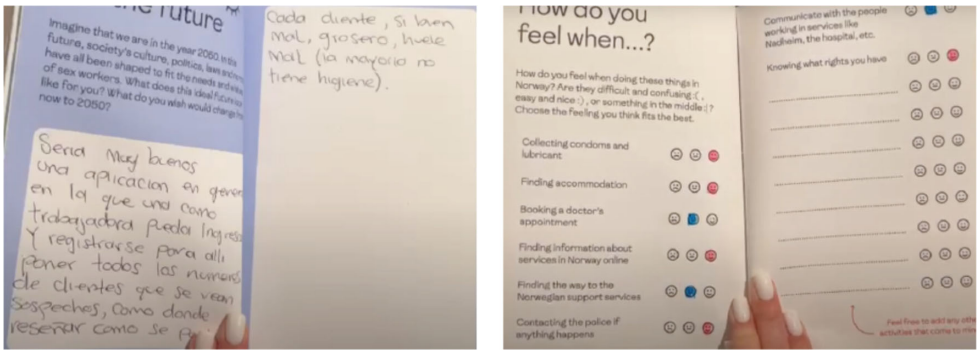



**Figure 3.** The cultural probe proved relevant for gaining insights into the lives of a woman who sells sexual services in Norway

Even professionals working within the system reported difficulty identifying appropriate services for their clients. There is no centralized overview of services available to PSST and therefore it is difficult for these individuals to know what is available in which city, or if they even are allowed to approach health services where they are. As a result, service providers rely on personal networks and informal knowledge to refer clients. Migrant PSSTs, which is the majority, who frequently travel between cities, face additional barriers due to regional inconsistencies in service availability. This fragmentation leads to inefficiencies, missed opportunities for care, and increased vulnerability for PSSTs. It also places a heavy burden on frontline workers, who must spend significant time locating and The students work further revealed that the actors perceive that while the persons selling sexual services need accessible and predictable health care and care in general, decision makers and research funding is focused on reducing or removing sex work, while **care was not on the agenda**:

> *Sex work is rarely on the agenda, and when it is, the focus is on how we can reduce it (NGO worker)*

> *I just think it's such a shame that we don't have a (health care) system for them (Nurse)*

Many of the conversations had were therefore about how to create something new that could ensure a focus on the care and wellbeing of persons selling sex rather than a focus on reduction. The focus on reduction may lead to further stigmatization.

**Health care access, inequality and the lack of social mobility** emerged as another critical area of concern. While PSST have the right to emergency and infectious disease treatment under Norwegian law, access to broader health services is uneven and often unclear. Many PSST are unaware of their health rights or fear seeking help due to stigma. Migrants without legal residency or health insurance face significant barriers. Moreover, testing and treatment for sexually transmitted infections (STIs) are inconsistently available across regions. To make matters worse, mental health support is severely lacking, despite high levels of stress, trauma, and isolation among PSST.

As one of the Nadheim workers expressed:

> *Some women come here to Nadheim on their first day of arrival to Trondheim looking well. Then a few months later they sit in front of us their whole body shaking*

> *(Nadheim worker)*

This quote shows the strong need for mental health support for persons who sell sexual services. They also explained that while they can try to suggest new work for PSST who want to exit the profession, there are few options once the CV only contains one kind of work, leaving many stuck in this type of profession and the social mobility is hence hard to achieve. Service providers further reported challenges in following up with PSST who test positive for STIs, especially when they have moved to another city. The lack of a national coordination system complicates treatment and increases health risks.

**Stigma, trust and relational barriers** were another pervasive theme across all data sources. PSST experience stigma from society, service providers, and even within their own communities. This stigma manifests in multiple ways: fear of being "outed" or identified as a sex worker; hesitation to seek help due to anticipated judgment or discrimination; internalized shame and self-stigmatization; hierarchies, also referred to as 'whorearchies', within the sex work community that create divisions and exclusion; These dynamics undermine trust in the service system and discourage PSST from accessing support. Service



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providers emphasized the importance of non-judgmental, relational approaches to care, but acknowledged that many institutions lack the training or awareness to provide such support. Results from the vignette study included 189 respondents in total, focusing on respondents under 30 years of age, show generally ambivalent attitudes toward PSST. Many respondents chose neutral options, indicating uncertainty and limited knowledge. They associated prostitution with social vulnerability and limited exit opportunities. Most did not attribute poor morals to PSST (78 % disagreed), yet only a small minority (14 %) believed that PSST have realistic possibilities of leaving prostitution. Women were generally more critical of prostitution than men, with twice as many women agreeing that it violates women's dignity. Overall, respondents were more negative toward prostitution as a phenomenon than toward the individuals involved, reflecting how stigma and moral ambiguity shape the context PSST navigate.

Finally, **Digital Exclusion and Information Gaps** were a key insight for the continued design process. The digitalization of sex work has created new challenges for outreach and support. While online platforms offer anonymity and flexibility, they also contribute to invisibility and exclusion.

Most PSST advertise and communicate through platforms like RealEscort, OnlyFans, and social media. Outreach workers struggle to contact PSST through these platforms due to privacy concerns and gatekeeping by agencies. They spend one human resource per office only for monitoring these websites to find the women that need talks or health care services, and each person selling services typically show up under various alter egos and phone numbers, where the phones are registered on facilitators.

Information about services is scattered across websites, often in Norwegian or English only, yet many of the workers speak only Spanish, Russian, Thai etc. Therefore, Nadheim often get better relations to the ones which languages they master themselves as service providers. They also explain that many PSST lack digital literacy or access to secure devices, and this can add to the digital exclusion and possibility to reach safe and correct information. This was also a challenge to the design study as the persons reached probably were only the ones that could read English or Norwegian well. Efforts to contact PSST via WhatsApp and other messaging services for the study yielded limited responses, highlighting the need for more inclusive and accessible communication strategies.

## 4. Design proposals: from feasible to speculative

The co-creative workshop with service providers combined with the other data gathered, helped identify the need of a centralized, accessible overview of services as the most obvious opportunity. Participants collaboratively drew, and designed the concept for a digital tool to support and strengthen both the persons selling services and the service providers offering care for them. Four key areas were selected, in collaboration with the service providers in the co-design workshop (see Figure 4).

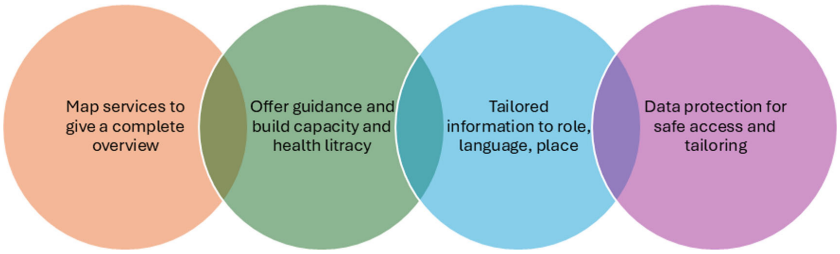



**Figure 4.** The four areas identified for designing "Ly" – a digital service to improve safety and health for PSSTs in Norway

The digital tool Ly was envisioned as a portal to the service system, reducing fragmentation and fostering trust. Participants were asked to sketch their dream solution and also highlighted the potential for Ly to improve coordination among service providers and to serve as a training resource. The Mock-ups were mostly hand drawn ideas that were discussed in the student design team and drawn in workshops with the service providers. The choices that emerged were the need for ;no login/no data storage need; many language options and finally the design of a simple, non-institutional interface tone and role-specific views (PSST vs. service providers etc.).



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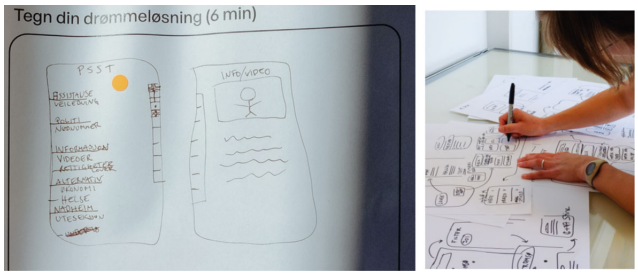



**Figure 5.** Sketches from co-design workshops, detailing and final interface showing how service providers and service users would reach each other in "Ly"

The students explored a deeper and more personal process, and the cultural probes and the vignette study were manifestations of this exploration. While the vignette study's data on stigma and attitudes did not provide direct data for any of the concepts, it inspired the students to develop a proposal that puts the PSSTs at the centre of a path towards more safety and autonomy. As a more radical, speculative suggestion the students also explored speculative design through a concept called the foundation of a new, fictional union for sex workers that challenges existing norms and imagines alternative futures. This critical fabulism allowed the designers to reflect on the limitations of current systems and the potential for transformative change outside the co-design format but as a reflection as a design team. This is a result of many hours of reflection, sketching and ideation, alongside the co-design process with stakeholders- as the students wanted to provide a feminist and situated proposal that can increase the autonomy of PSSTs.

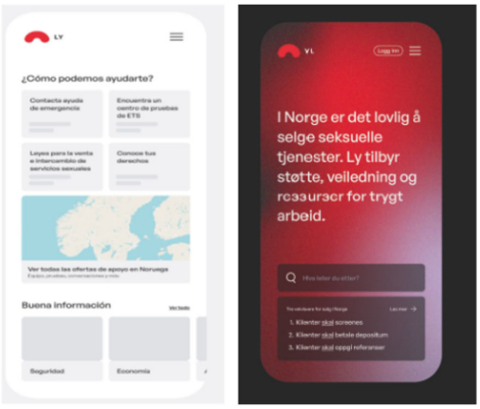



**Figure 6.** The service access tool Ly (left) and the speculative, union forming dark mode Yl (right)

In their suggestion "Dark modus" the students explore the hope that something larger and more bottom-up can be nudged through a platform, turning the title from LY to YL. If we envision a future in which the prevailing problem narratives surrounding prostitution evolve in a different direction, it becomes possible to design services that diverge significantly from those we have previously proposed. In this more radical version, the students consider more of the concerns voiced directly by the PSSTs, where they can report clients so that other PSSTs are warned about them, they can get help to send money abroad, and more importantly, they can get in touch with each other and form an underground union. This is a more speculative proposal because it can conflict with legal concerns and contain possible risks. Yl emerged from PSST-voiced concerns about: unsafe clients lack of peer networks dependence on facilitators desire for collective strengthShould the issues cease to revolve solely around rescuing individuals from prostitution and instead concern wages, labour rights, and market regulation, new opportunities emerge. By assisting PSSTs in organizing, we can foster collective solidarity and challenge the market forces that enable the exploitation of individual workers. Although we remain intellectually unsettled regarding the broader questions of sexual service transactions, it is unequivocal to us that PSST requires enhanced rights and improved, safer working conditions. Consequently, the tool's dark mode assumes the form of a labour union. While Ly is a digital tool for service providers, the speculative version Yl significance lies in its potential to catalyse broader change. By making the service system more visible, navigable, and humane, it challenges the status quo and invites reflection. It also creates opportunities for collaboration between actors who rarely interact, such as police, health workers, and outreach organizations. Further work is needed to develop and implement Ly in collaboration with national health and justice authorities.



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## 5. Discussion

The findings of this study reveal a deeply fragmented and stigmatized service system surrounding sex work in Norway. Through a relational design approach, these challenges are not merely logistical or technical—they are relational, systemic, and political. This section discusses how relational design, feminist theory, and systemic thinking can help reframe the problem space and guide more inclusive, ethical, and effective interventions. Relational design emphasizes the importance of designing through relationships, empathy, and mutual recognition. In this project, relational design was not a method but a stance—a commitment to engaging with PSST and service providers as co-creators, not subjects. This approach allowed the design team to build trust, uncover hidden needs, and co-develop solutions that reflect lived realities. The concept of Ly, a digital tool for navigating the service system, emerged directly from these relational encounters. It was not imposed from above but shaped through dialogue, workshops, and iterative feedback. This aligns with Nielsen and Bjerck's assertion that relational design is about "designing with care," where the process itself becomes a form of support and recognition.

### 5.1. Co-design and the politics of visibility

Feminist design theory challenges designers to confront entrenched power structures and center marginalized voices (Place 2023). Within co-design, the politics of visibility requires creating tools and spaces that safeguard confidentiality and agency, enabling sex workers to decide when and how they are represented(Light & Akama 2014). Engaging with invisible or marginalized communities – such as sex workers – introduces profound ethical and political considerations that feminist designers actively embrace rather than overlook. In short: visibility is political because it determines who gets recognized, who gets surveilled and who gets silenced. For sex workers, it's a constant negotiation between empowerment and exposure. In this study, sex workers were involved with care to strengthen their political demands. The design process itself became an empowering vehicle, fostering horizontal empathy and collaboration among marginalized citizens. By focusing on care and inclusion, feminist design reframes participation as a means of collective agency and social transformation. In the context of sex work, this means resisting dominant narratives that portray sex workers as either victims or deviants, and instead recognizing their agency, complexity, and humanity. An interesting finding is that while the PSSTs need visibility to sell and to be able to work, their clients are hidden and so are the middlemen profiting and having power. If these are made visible by the design proposal, legal concerns are raised, illustrating the power that lies in visibility and invisibility. The student project's use of cultural probes, autoethnography, and participatory workshops reflects this feminist ethos. These methods allowed PSST to share their experiences on their own terms, without being reduced to data points or stereotypes. The vignette study further revealed how public attitudes toward sex work are shaped by moral judgments and gendered assumptions, underscoring the need for design interventions that challenge stigma and promote dignity. Moreover, the speculative concept Yl, a fictional union for sex workers, exemplifies feminist design's capacity for critical fabulism. By imagining alternative futures, the designers were able to question existing norms and explore possibilities for collective empowerment, solidarity, and systemic change.

### 5.2. Systems oriented design and the challenge of complexity

System-oriented design (SOD) provided the analytical tools to map the complex ecosystem surrounding sex work in Norway. The gigamps and actor diagrams revealed a system characterized by fragmentation, opacity, and uneven access. These visualizations helped stakeholders see the connections—and disconnections—between services, laws, and lived experiences. However, SOD also revealed the limitations of traditional service design approaches. The challenges faced by PSST are not isolated problems but symptoms of deeper structural issues: criminalization, migration policy, digital exclusion, and institutional stigma. Addressing these requires not just better services, but systemic transformation. The design of Ly attempts to bridge this gap by offering a tool that is both practical and systemic. It provides immediate support—such as locating health services or understanding legal rights—while also highlighting the need for better coordination, training, and policy reform. The Giga-mapping allowed for identification of both the feasible (Ly) proposal and the need for a more speculative suggestion (YL).



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### 5.3. Trust, stigma, and the need for pluralistic design

Trust and stigma emerged as central themes in the research. Many PSSTs avoid services due to fear of judgment, exposure, or discrimination—fears grounded in real experiences of exclusion. Relational design responds by prioritizing safety, anonymity, and respect. The ethical design of Ly reflects this through privacy-by-design and "consentful technology" principles (Lee, 2023): no tracking, multilingual support, anonymous access, and resources to help service providers reduce stigma.

The diversity of PSSTs also challenges assumptions of a "universal user." Following feminist scholars such as Haraway and Costanza-Chock, identities are intersectional, situated, and shaped by overlapping factors like gender, race, migration status, language, and class. Designing for this complexity requires pluralism—adaptable systems rather than one-size-fits-all solutions. The proposed application and speculative workers' union attempted to embody this by tailoring information to users' roles, languages, and locations, acknowledging that, for example, a trans migrant sex worker in Oslo may have different needs from a Norwegian citizen working in Trondheim. This pluralistic approach extends to service providers as well, whose knowledge and capacities vary widely. While Ly offers tools to support more informed, compassionate services, it is ultimately the methodological choices and sensitivities of the design facilitators that make the work "relational," not just the resulting artifacts.

## 6. Key conclusions

The described design process represents a rich case of what Relational Design can look like in practice. The students navigated complexity and tried out experimental and designerly approaches to put themselves in the perspective of PSSTs with high income, migrants and also persons in the city with attitudes and perceptions. They tried to really spend time in each of these partial viewpoints through a variety of lenses to free themselves from predisposed problem understandings. The project demonstrates that design can be more than problem-solving—it can be system-shifting. Through relational, feminist, and systemic design, it is possible to illuminate hidden structures, amplify marginalized voices, and co-create pathways toward justice. This study has explored how relational design, feminist theory, and systemic service design can be applied to address the complex challenges faced by people who sell sexual services (PSSTs) in Norway. Through a collaborative process with Nadheim Trondheim and other stakeholders, the project uncovered critical gaps in the service system, including fragmentation, lack of coordination, digital exclusion, and pervasive stigma. The concept of Ly, a digital tool co-designed with service providers, represents a tangible response to these challenges. It offers a centralized, accessible, and anonymous portal to health services, legal information, and support networks. More than a technical solution, Ly embodies the principles of relational design—it is built on trust, empathy, and co-agency. It seeks not only to improve access but to foster dignity, safety, and recognition for PSST. We would suggest expanding relational design practices to other marginalized groups and service systems. Ultimately, this research affirms that sex workers, like all people, deserve respect, safety, and access to their rights. Whether they work independently or within networks, whether they are migrants or citizens, whether they sell services online or in person—they are part of our communities. Design, when practiced relationally and ethically, can help ensure that they are seen, heard, and supported.


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